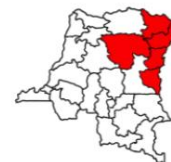




EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°112/MVEBDB/03/09/2026

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HIGHLIGHTS (last 24 hours)

As of September 3, 2026, 94 new confirmed cases of Ebola Virus Disease (EVD), including 17 community deaths, were recorded in the provinces of Ituri (60 cases), North Kivu (25 cases), and Haut-Uélé (9). In addition, 6 intra-CTEs have been recorded.

No news

Health zone affected in the last 24 hours with a 31.3% decrease in overall alert notification.

ACCUMULATIVE CASE 6,436	ACCUMULATIVE DEATH CONFIRMED 3,095	LETHALITY 48.1%	PATIENTS IN ISOLATION/CTE 738	ACCUMULATIVE HEALED 1,495	TRACKING RATES CONTACTS (Of the Day) 88.5%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



60 out of 151 health zones affected (39.7%)



Summary of key points from the last 24 hours

Over the past 24 hours, there has been sustained notification with **94**

New confirmed cases, including **17 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (60 cases) and North Kivu.**

(25) and Haut Uélé (9). However, **6 intra-CTE deaths** were recorded, including 3 in Ituri, 2 in North Kivu and 1 in Haut Uélé, bringing **the number of deaths for the day to 23.**

In addition, **20 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (18 in Ituri and 2 in North Kivu).

From the start of the epidemic until September 3, 2026, **6,436 confirmed cases, including 3,095 deaths**, were reported in the DRC, representing a case fatality rate of **48.1 %**.

The epicenter remains the Ituri province, accounting for **81.2%** of confirmed cases recorded in the country and **63.8%** of newly reported confirmed cases over the past 24 hours.

In the six affected provinces, **60 out of 151 health zones (39.7%)** remain affected. Ituri with 28 health zones affected out of 36, followed by North Kivu (15/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 3, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	60	5224	2,355	45.1%	28/36 (77.8%)
North Kivu	25	938	623	66.4%	15/34 (44.1%)
Haut-Uélé	9	245	104	42.4%	6/13 (46.1%)
Tshopo	0	22	9	40.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	94	6,436	3,095	48.1%	60/151 (39.7%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 3, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5224	2355	45.1%	60	10	3	13
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	170	36	21.2%	2	0		0
Boga	2	2	100.0%				0
Bunia	1455	449	30.9%	11	2		2
Damascus	30	9	30.0%	2	0		0
Drodro	14	7	50.0%				0
Fataki	77	30	39.0%				0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	98	66	67.3%	6	4		4
Lita	231	121	52.4%	6	0		0
Logo	12	5	41.7%	1	0		0
Lolwa	22	5	22.7%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	36	21	58.3%				0
Mangala	278	138	49.6%	7	0		0
Mongbwalu	623	288	46.2%	7	0		0
Nia-Nia	221	113	51.1%	3	0		0
Nizi	683	267	39.1%	8	1		1
Nyankunde	124	35	28.2%				0

Rimba	11	4	36.4%				0
Rwampara	977	344	35.2%	6	3		3
Tchomia	60	25	41.7%	1	0		0
To ventilate	N / A	354	N / A	N / A	N / A	3	3
North Kivu	938	623	66.4%	25	7	2	9
Blessed	156	115	73.7%	5	5	1	6
Good	1	1	100.0%				0
Butembo	193	136	70.5%	7	1	1	2
Goma	1	0	0.0%				0
Kalunguta	17	8	47.1%	1	1		1
Katwa	427	278	65.1%	11	0		0
Kyondo	16	12	75.0%				0
Lubero	4	3	75.0%				0
Mabalako	7	5	71.4%				0
Manguredjipa	2	1	50.0%				0
Masereka	13	5	38.5%	1	0		0
Musienene	85	48	56.5%				0
Mutwanga	2	2	100.0%				0
Oicha	8	6	75.0%				0
Vuhovi	6	3	50.0%				0
Haut-Uélé	245	104	42.4%	9	0	1	1
Boma	29	14	48.3%	2	0		0
Mangbetu	3	1	33.3%				0
Gombari	3	1	33.3%				0
Isiro	89	36	40.4%	3	0	1	1
Pawa	39	22	56.4%				0
Rungu	2	1	50.0%				0
Wamba	83	30	36.1%	4	0		0
Tshopo	22	9	40.9%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6436	3095	48.1%	94	17	6	23

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 3, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	20	4	24	18	3	2	1	21	8
Bas Uélé	39	1	40	16	1	23	0	17	0
Ituri	659	81	740	140	53	280	6	193	126
North Kivu	808	41	849	70	39	719	2	109	56
South Kivu	ND	ND	N / A	ND	ND	ND	ND	ND	ND
Tshopo	139	0	139	5	0	134	0	5	0
Total General	1665	127	1,792,249	96	1158	9	345	190	

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

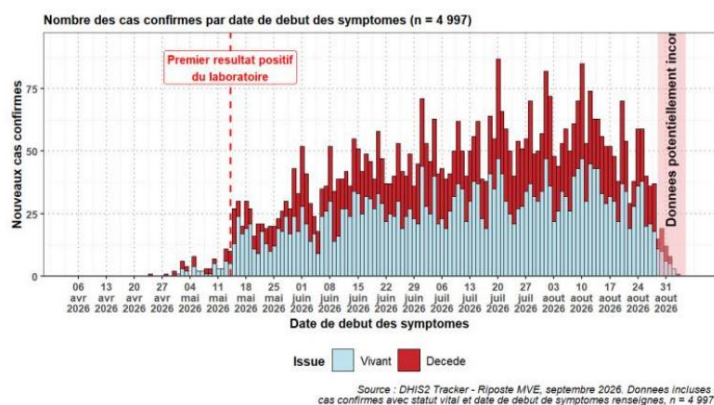
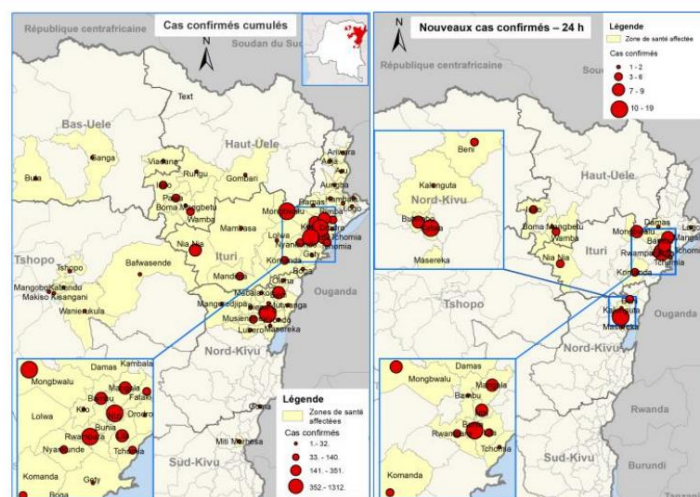


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

As of September 3, 2026, 1,792 alerts have been recorded in the 5 provinces of those reported (South Kivu did not report), 1,512 (84.4%) were verified. Thus, 345 alerts were validated as suspected cases. All of these were investigated (100%) and 190 of them were transferred to CT/CTE (Table 3).

A slight improvement was observed in the proportion, namely 88.5% (21,678 views out of 24,497). to be continued) unlike the previous day (86.5%) and remains above the threshold of 85%; 85.1% in North Kivu (7,712/9,060), 91.1% in Ituri (12,173/13,362), 92.1% in Haut Uélé (1,449/1,573), 69.6% in Bas Uélé (103/148) and 68.1% in Tshopo (241/354).

1.1.3. Challenges

Persistent low contact tracing around confirmed cases in the Lower Province Uélé, linked to a lack of teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

Ten (10) alerts were notified to the various PoE/PoCs as of September 3, 2026, two of which were validated in Ituri: a live alert at the Malabo PoC (Nyankunde Health Zone), transferred to the Nyankunde CTE, and a body intercepted at the Mudzibala PoC (Bunia Health Zone), for which the EDS is underway. In North Kivu, four of the five alerts were validated and referred (Mususa, Maboya, and Mukulia PoCs). No alerts were notified in Haut-Uélé, where the Magambe and Aero Matari PoCs were equipped with loudspeakers and megaphones by IOM. The three alerts from Bas-Uélé have been invalidated.

Table 4. Status of key indicators for PoC/PoE by province, as of September 3, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé		Overall
Report completeness	92%	100%	55.5%	83%		64%	ND	86.3%
% of functional strategic PoE/PoC	9.1%	N / A	N / A	N / A	N / A	N / A	N / A	9.1%
Number of people who have switched to PoE/PoC	128,085	85,790	8,848	14,924	11,832	5,690		255,169
% of travelers screened: 98.0% 98.0% 98.9% 99.0%	98.0%	98.0%	98.9%	99.0%				
% of travelers who washed their hands	98.0%	98.0%	98.9%	99.0%			ND	98.1%
% of travelers made aware	99.0%	98.0%	100%	100%			ND	98.7%
Number of alerts notified	2	5	0	0	0	0	3	10
Number of live alerts validated (suspected cases)	1	4	0	0	0	0	0	5
Number of bodies recovered today	1	0	0	0	0	0	0	1
% of suspected cases transferred to CT/CTE	100%	100%	N / A	N / A	N / A	N / A	N / A	100%
% of lifeless bodies swabbed/secured	0%	N / A	N / A	N / A	N / A	N / A	N / A	0%
% of suspected cases investigated within 24 hours	100%	100%	N / A	N / A	N / A	N / A	N / A	100%

1.2.2. Challenges

Operationalization of PoE/PoC is suboptimal, and reporting is low in South Kivu (55.5%) and Haut-Uélé (64.0%). In Ituri, only 1 (9.1%) of the 11 strategic PoE/PoCs remains fully operational: the Avakubi PoC remains non-functional for security reasons, and the Nzebi, Frigo, Galay, and Bey PoCs are on strike due to non-payment. In Haut-Uélé, 4 PoCs (Dusu, Apodo, Aungba, and Gossamu) have not reported, and the Magambe PoC has recorded 209 screening refusals (5%). In South Kivu, the lack of isolation devices at the Kavimvira, Kalundu Port SNCC, and Baraka PoEs persists. In Tshopo, 18 out of 30 planned PoCs are activated, and in Bas-Uélé, insufficient mobility and connectivity remain.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 60 new positive results** (50 living and 10 dead) out of 287 new samples received and analyzed (**positivity: 20.9%**).
- **North Kivu : 25 new positive results** (18 living and 7 dead) out of 176 samples analyzed (**positivity of 14.2%**).
- **Haut-Uélé: 9 new positive results** (9 live) out of 37 samples analyzed (**positivity: 24.3%**).
- **Bas-Uélé: 8 samples** received and tested (5 live and 3 dead), **all returned negatives**.
- **Tshopo: 6 samples** received and tested for analysis (6 live), **all came back negative**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 20 treatment centers were opened around the 27 cases addressed by the pillar (74.1%), 140 households were decontaminated out of 151 identified (92.7%), and 7 out of 9 health facilities/social welfare centers were decontaminated (77.8%); 15 infection prevention and control (IPC) kits were distributed to households. Regarding the health data collection (HDC), 83 alerts were recorded, including 10 postponements from the previous day, 62 HDCs were carried out, and 48 bodies were swabbed.
 - **In North Kivu**, the EDS pillar received 46 alerts, carried out 41 EDS and swabbed 45 out of 46 bodies (97.8%), the only failure being linked to community resistance in the AS of Mataba (ZS Kalunguta); 21 cases are postponed to the following day.
 - **In Tshopo**, no infection control ring could be opened around the 20th case due to community resistance, despite the involvement of the area's chief medical officer; 5 rooms at Kabondo General Referral Hospital were decontaminated. Monitoring and support covered 13 infection control centers (ICCs) in 5 health zones (Makiso-Kisangani, Kabondo, Tshopo, Lubunga, and Mangobo), with briefings for 51 service providers and the installation of 7 triage units; 46 of the 79 ICCs under surveillance (58%) have infection control kits and a triage system.
- In Bas-Uélé**, activities focused on setting up a functional triage at the Buta General Referral Hospital, briefing 23 BCZ members including 6 hygienists in Likati, briefing 38 service providers and 45 hygienists in Buta and Viadana, as well as the PCI assessment of 4 ESS in the Viadana Health Zone (Viadana General Referral Hospital 47%, Limba Health Center 40%, Mbana Health Center 21% and Vungba Health Center 14%).

1.4.2. Challenges

- Weak implementation of infection prevention and control (IPC) standards: In Ituri, ring closures remain incomplete (74.1%, 20/27) and decontamination teams remain insufficient in Fataki, Mangala, and Nizi; In Bas-Uélé, no rings have been opened or households decontaminated, 6 health zones (Buta, Ganga, Viadana, Titule, Poko, and Bondo) remain untrained in IPC, and two referral hospitals (Ganga and Viadana) lack functional triage. In Tshopo, community resistance is blocking the opening of the ring closure for the 20th case, IPC supplies remain insufficient in health centers (only 1 out of 79 health centers has a score above 80%), and the incinerator at Makiso-Kisangani Referral Hospital remains out of service.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 53 new admissions to the various dedicated facilities, compared to 66 discharges, including 18 recoveries. Thus, 434 patients are hospitalized out of 978 beds, representing an occupancy rate of 44.4%.

- **In North Kivu**, 70 new admissions and 78 discharges were recorded, including 2 recoveries; 225 patients are hospitalized expressing an overall bed occupancy rate in CTE/CT is in oversaturation (102.7%; 220 beds available).
- **In Haut Uélé**, there were 14 new admissions and 7 discharges (4 non-cases, 2 suspected deaths, and 1 escape of a confirmed case). Thus, 62 patients are hospitalized (49 confirmed and 13 suspected) out of 128 beds, representing an occupancy rate of 50.8%.
- **In Tshopo**, no new admissions were recorded and one confirmed case escaped on day 4 of hospitalization. Six (6) patients are in isolation out of 25 beds, representing an occupancy rate of 24.0%.

1.5.2. Challenges

Low capacity and overcrowding: In North Kivu, CT/CT facilities remain overcrowded (102.7%), and more than half of patients (51.7%) are treated in non-standardized healthcare facilities; in Ituri, the Lita CT facility is saturated with suspected cases, and the expansion of the Nizi and Bambu sites is delayed. In Tshopo, the unavailability of safe blood has persisted for six days, and the technical infrastructure remains insufficient for biochemical and hematological tests; in Bas-Uélé, the lack of a standardized CT/CT facility and ambulances continues.

1.6. Vaccination 1.6.1.

Main actions

Vaccination activities that began in the provinces of Bas-Uélé and Tshopo have enabled the vaccination of frontline workers and/or service providers. A total of 1,834 people were vaccinated with the ERVEBO vaccine, including 1,375 in Tshopo and 459 in Bas-Uélé.

1.6.2. Challenges

Lack of mobility for vaccination teams in advanced strategy in the Haut Uélé province;

1.7. Continuity of Care

In Bas-Uélé, support to laboratory and surveillance teams for the preliminary investigation of the increasingly frequent deaths in the Buta health zone (SS and community); strengthening of communication on compassionate treatment for Ebola.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, the CREC supported the response around the latest confirmed cases in the health zones of Nia-Nia (AS Juhudi), Nizi (AS Heritage) and Lolwa (AS Mabangifo), affecting 1,357 people including 76 women, with the listing of 72 contacts; a mass awareness campaign was conducted at the place of mourning in Nia-Nia and the chieftaincy of Bahema Banywagi was mobilized in the AS of Motumbi (ZS Lita).
- **In North Kivu**, 5,980 home visits were conducted, reaching 17,624 people; 58 educational talks were held with 982 people; 3 briefings (63 people), 2 advocacy sessions, 3 community dialogues (80 people), and 3 mass communication campaigns (1,690 people) were organized; 3 feedback and infodemics were resolved within 48 hours out of 7, and 153 alerts were reported by community actors. Interventions in schools have begun in hotspot areas.
- **In Tshopo**, 1,049 people were sensitized (611 women and 438 men); 2 refusals lifted out of 3 identified, 2 rumors clarified and 2 community feedbacks collected; 211 leaflets and 5 banners distributed and 3 articles published by online media; support for the supervision of vaccination of PPL against Ebola virus disease and continued training of community relays).

- **In Bas-Uélé**, briefing and awareness-raising of 50 youth representatives from Buta and closing of the training session of 160 young people, members of civil society, media professionals and motorcyclists with the support of FHI 360; advocacy with the sub-division of EPST of Dingila for the observance of preventive measures in schools, awareness-raising on EDS at the market and at the University of Monga; intensification of community-based surveillance in the Aketi Health Zone.

1.8.2. Challenges

Persistent community resistance: in Ituri, 21 refusals of isolation at the CTE in the health zones of Rwampara, Bunia, Tchomia and Lolwa and 9 bodies not swabbed due to resistance in Bambu, Fataki and Mandima; in North Kivu, reluctance to decontaminate a household in the health zone of Katsya (Butembo health zone) and aggression against the team from the health zone of Kalunguta in the health zone of Mataba; in Tshopo, resistance blocking the opening of the ring of the 20th confirmed case and persistent resistance to investigations, sampling and transfers to the CTE.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In North Kivu**, 25 new confirmed cases and 139 of the 204 suspected cases benefited from SMSPS interventions; 79 of the 100 results were announced, including 20 positive results, 45
In Haut-Uélé, 24 confirmed cases received support, 12 results announced (4 positive), 62 support workers, and 34 supported PPLs. In Tshopo, 1 confirmed case and 2 suspected cases received support, and 2 recovered patients reintegrated into the response.

1.9.2. Challenges

In Haut-Uélé, there is a shortage of SMSPS staff in Boma Mangbetu and 19 recovered patients left the Isiro CTE without a kit; in Tshopo, there is an absence of service providers in 3 health zones.

1.10. Logistics

1.10.1. Main actions

- **In Ituri**, work continues on leveling the site of the Mangala CTE and organization inter-CTE transfers.
- **In North Kivu**, 10 calls received resulted in 10 successful trips (100%). In Tshopo, the INSP received IPC supplies and construction continued on the Madula isolation site.
- **In Bas-Uélé**, support for the deployment of the surveillance team for contact tracing, alert verification and active research, and continuation of vaccination of PPL and TPL in the Buta health zone.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, security support for EDS teams for the burial of two confirmed cases and one community death, and coverage of PCI teams in Mabolio and Ngongolio.

1.10.4. Challenges

Twenty-one (21) refusals to self-isolate at the CTE were reported in Ituri. In North Kivu, officials resisted compliance with the health protocol at the PoCs, and security was reinforced at the Vulindi PoC (Butembo Health Zone).

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri, 75

agents were briefed on the prevention of sexual exploitation, abuse and harassment and signed the code of good conduct (ZS Bunia, Aru and Mambasa).

1.11.2. Challenges

- Still insufficient coverage of PSEA activities; lack of stakeholder mapping; insufficient mass awareness materials; lack of transport and IT equipment for the pillar.

KEY MESSAGES

- Performance achieved in epidemiological investigations (**100% of suspected cases investigated**) is being consolidated and the daily proportion of contact tracing (**88.5%**) remains stable and above the threshold of 85.0%.
- A total of **94 new confirmed cases**, including **17 Community deaths and 6 intra-CTE deaths** were recorded in the last 24 hours, bringing the national total to **6,436 confirmed cases and 3,095 deaths**.
- The ongoing notification in the provinces of Ituri, North Kivu and Haut-Uélé highlights sustained community transmission of the disease.
- Ituri remains the epicenter of the epidemic, concentrating **81.2 %** of cumulative confirmed cases and **63.8%** of new cases reported in the last 24 hours, while North Kivu province holds 26.6% of new cases with the highest lethality.

- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a national and then continental public health emergency, accompanied by mobilization funds.



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